

REGULATORY & COMPLIANCE

Practice Compliance Calendar & Renewal Tracker

One tracker for every license, registration, policy and annual task, an annual calendar, and a record retention and destruction log.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Tracker + calendar

VERSION

1.0 · Sept 2026

TIME TO ADAPT

1 hour to set up

WHAT IT IS

A single place to see every date that could cost you your license, your billing or your peace of mind: renewals, attestations, training, reviews and records retention.

USE IT WHEN

You hold more than one license or registration, are new in practice, or just missed a renewal.

CUSTOMIZE FIRST

Every date and number in the tracker. Lead times depend on your board and payers, so check them against your own sources.

How to use it

1. Pick one recurring “compliance day” each quarter and put it on your calendar. Half an hour is enough when the tracker is current.
2. Enter every item once, with the expiry date and the lead time it needs. Set reminders at 90, 60 and 30 days before each expiry.
3. Do the hard ones first. Renewals that require continuing education, background checks or supporting documents take longer than the form.
4. Record when you completed each task and where the proof is saved. Keep certificates and confirmation emails in one folder.
5. Review the tracker at each quarterly check (Part D) and after any change of address, employer, payer or license.

Part A: Master renewal tracker

The rows below are the items most practices have. Add your own. Cycles shown are typical and vary, so confirm your own.

Item	Typical cycle	Number / ID	Expires	Lead time	Proof saved
License, state 1	Every 1 to 3 years, board-specific			Start [90] days ahead	
License, state 2 (each state where you treat)	Board-specific				
Continuing education hours and required topics	Tied to license renewal			Complete [60] days ahead	
Malpractice policy	Annual			[60] days	
Malpractice tail or prior-acts coverage (if applicable)	On policy change				
DEA registration (prescribers)	Every 3 years			[90] days	
State controlled-substance registration (prescribers)	State-specific				
NPI record (NPPES)	Update within 30 days of a change; review yearly				
CAQH profile re-attestation	Every 120 days			Set a repeating reminder	
Payer recredentialing	Commonly every 2 to 3 years			[120] days	
Medicare or Medicaid enrollment revalidation	Commonly about every 5 years				
PSYPACT E.Passport and APIT / TAP	Annual (E.Passport and APIT)			See the Telepsychology Consent	
Board certification or maintenance of certification	Program-specific				
Cyber liability and general liability insurance	Annual			[60] days	
Business license, entity annual report, registered agent	Annual				
Professional memberships and dues	Annual				

Part B: Annual calendar

A suggested spread of tasks. Dates depend on your fiscal year and your board's cycles. Make it yours.

When	Task	See
January	Send or file year-end tax forms for contractors and staff. Review malpractice, cyber and general liability renewals. Check the status of federal telehealth prescribing rules, which are set to expire December 31, 2026 unless extended (prescribers).	Prescribing kit
February to March	Log any HIPAA breaches from the prior year and report those affecting fewer than 500 people to HHS within 60 days of year-end. Complete the annual HIPAA and security training. File year-end paperwork.	Security analysis
April	Federal estimated tax payment. Refresh your risk analysis if systems changed. Review consent and privacy forms for legal changes.	Consent, privacy
June	Second estimated tax payment. Mid-year fee and payer-mix review. Test your backup restore. Review AI vendor terms and BAAs.	AI policy, security
July to August	Update your professional will and the coverage arrangements before autumn. Check CE progress against your renewal date. Send new Good Faith Estimates for continuing self-pay patients before their 12-month period ends.	Will, financial policy
September	Third estimated tax payment. Start planning any fee increase and give patients at least 30 days' notice. Check DEA, HIPAA and PSYPACT news.	Financial policy
October	Inventory patients on controlled medication by telemedicine and plan for in-person care if needed (prescribers). Review state and payer telehealth rules for the coming year.	Prescribing kit
November to December	Record retention review and secure destruction (Part C). Send fee-change notices. Year-end review of documents, licenses and registrations. Renew memberships. Update this tracker for next year.	This tracker

CAQH AND NPES ARE THE TWO MOST OFTEN MISSED

CAQH asks you to re-attest about every 120 days, and payers can suspend claims if the profile lapses. NPES asks you to update your NPI record within 30 days of a change in name, address, taxonomy or other core details. Put both in a repeating reminder.

Part C: Record retention and destruction

How long you must keep records comes from state law, your license type, your payer contracts and your malpractice carrier, and the longest period wins. The APA's record keeping guidelines suggest keeping adult records for seven years after the last professional contact as a common minimum. Records of minors are usually kept longer, often until the age of majority plus a set number of years. Legal holds override all schedules: never destroy records that may be part of a claim, a complaint or a subpoena.

Question	Your answer
State law and board rule on adult records	
State law and board rule on minors' records	

Question	Your answer
Payer contract or carrier requirement	
My retention period (the longest of the above)	
Where records are kept and who is custodian if I am unable to practice	
Method of secure destruction and vendor (BAA if applicable)	

Destruction log

Patient ID (not name)	Last contact	Retention end	Legal hold check	Method and date	Initials

Part D: Quarterly 30-minute compliance check

- ☐ All items in the tracker reviewed. Anything expiring in 120 days has a plan.
- ☐ Malpractice, cyber and registration details still correct after any change of address or employer.
- ☐ Continuing education on track. Certificates saved.
- ☐ Backups verified. Software updates applied. Departed staff and colleagues have no access.
- ☐ BAAs current for every vendor holding patient data.
- ☐ Any incidents, complaints or near misses recorded and reviewed.
- ☐ New laws, board rules or payer changes reviewed. Consent forms and notices updated where needed.
- ☐ Coverage arrangements and professional will still accurate.

IN PSYALLIANCE

The credentials panel in your profile tracks license expiry, CE hours and insurance. Treat this document as the wider checklist and enter your license dates there so the dashboard flags them for you.

Sources and further reading

- CAQH ProView attestation requirement (caqh.org).
- CMS, NPPES: update NPI information within 30 days of a change (nppes.cms.hhs.gov).
- DEA Diversion Control Division, registration renewal (deadiversion.usdoj.gov).
- PSYPACT Commission, E.Passport and Authority to Practice renewals (psypact.gov).
- APA Record Keeping Guidelines (apa.org/practice/guidelines/record-keeping).
- HHS, Breach Notification Rule, annual reporting of breaches affecting fewer than 500 individuals (hhs.gov/hipaa).

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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